advancement of pharmaceutical sciences and public health. The NBFC's integrated approach is pivotal as we face challenges of urbanisation and environmental degradation, focusing on enhancing biodiversity, fostering green jobs, and improving global health by integrating biodiversity and human wellbeing indices. Aligned with the European Resilience Plan and focusing on the youth, the NBFC is committed to ensuring future generations live in harmony with nature and achieve the Sustainable Development Goals. We trust that this Correspondence will underscore the importance of biodiversity in safeguarding not only our planet's health, but also the health and prosperity of its inhabitants.

We declare no competing interests. We extend our heartfelt gratitude to Giuseppe Remuzzi not only for sharing our vision and concerns, but also for his invaluable revision and unwavering support. The research is funded under the National Recovery and Resilience Plan by the European Union–NextGenerationEU (NBFC: Project code CN00000033; CUP: F13C22000720007).

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See **Online** for appendix

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Immunotherapy in frail non-small-cell lung cancer patients

We read the study published by Siow Ming Lee and colleagues[1] with great interest. The authors concluded that atezolizumab monotherapy was associated with better outcomes for patients deemed ineligible for platinum-based chemotherapy. However, the study raised two substantial concerns regarding the heterogeneity of the study population and the single-agent treatment of the control group.

First, the trial combined frail and very frail patients with an Eastern Cooperative Oncology Group performance status (ECOG PS) 2 and 3 with a substantial number of fit (ECOG PS 0–1) older patients. These are three very distinctive groups who respond differently to treatment.[2] There was no difference between immunotherapy or chemotherapy in ECOG 2 (hazard ratio [HR] 0·86, 95% CI 0·67–1·10). The paper even stated that the median overall survival of ECOG PS 2 patients treated with chemotherapy was better than those treated with immunotherapy (9·7 *vs* 10·4 months). The question here is whether the benefit of the total group might be driven by the fit older patient ECOG PS 0–1 group.

Second, IPSOS investigators possibly deemed platinum-based therapy unsuitable for patients primarily by their performance score. A 2023 Cochrane review, with IPSOS data included, showed that patients with ECOG PS 2 should be treated with platinum doublet therapy first, not non-platinum monotherapy (HR 0·67 [0·57–0·78]), contrary to its use in this trial.[3] Furthermore, the observed crossing of the survival curve during the first months could be attributed to withholding chemotherapy in a substantial part of the atezolizumab group. Consequently, the findings from this study for patients with ECOG PS 2, should be interpreted with caution.

WHvG has unpaid roles for the European Respiratory Society and Dutch Association of Physicians for Pulmonary Diseases and Tuberculosis outside of the submitted Correspondence; and is a local investigator for clinical trials for MSD and Roche. HG declares grants or contracts from Roche and Boehringer Ingelheim; and consulting fees from Lilly, Novartis, Roche, Genentech, and AstraZeneca. All other authors declare no competing interests.

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Comprehensive inclusion: demographics of clinical trials

We read with great interest Siow Ming Lee and colleagues'[1] study on the efficacy and safety of atezolizumab in patients with non-small-cell lung cancer. The research commendably addresses a broader demographic of lung cancer patients who are less commonly included in clinical trials compared with patients who have solid organ tumours.[2] But an unmistakable concern arises: the grave under-representation of Black patients.